

February 23, 2026

The Honorable Thomas Keane
Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information
Technology
Department of Health and Human Services
Attention: HHS Health Sector AI RFI
Mary E. Switzer Building, Mail Stop: 7033A, 330 C Street SW
Washington, DC 20201

Submitted electronically at <https://www.regulations.gov>.

RE: Response to Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as
Part of Clinical Care

Organization Overview

Hospital for Special Surgery (HSS) is the world's leading academic medical center focused on musculoskeletal health and the oldest orthopedic hospital in the United States. HSS has been ranked number one for orthopedics for 16 consecutive years by U.S. News & World Report and has been among the top-ranked hospitals for both orthopedics and rheumatology for 34 consecutive years. HSS combines clinical care, research, education, and digital innovation to improve patient outcomes at scale.

HSS is focused on innovating in healthcare AI, advanced analytics, and digital transformation to support clinical excellence, operational efficiency, and patient engagement. Our work integrates secure cloud infrastructure, enterprise data platforms, predictive analytics, and AI-enabled workflows into real-world clinical environments.

Current examples of focus include:

- Machine learning models
- Predictive analytics for outcomes, complications, and utilization
- Natural language processing to extract structured insights from clinical documentation
- AI-assisted operational workflows to reduce administrative burden
- Data-driven quality improvement and performance monitoring
- Secure cloud architecture aligned with HIPAA and healthcare cybersecurity standards

HSS approaches AI as applied innovation. Solutions are designed for real clinical workflows, validated with physicians and care teams, and governed through structured compliance, privacy, and model oversight processes. To support this, HSS has established an AI Governance Subcommittee within our Data Governance structure. This group reviews proposed AI solutions, sets standards, and monitors ongoing performance with a focus on safety, transparency, and fairness. Embedding AI oversight in our data governance framework ensures systematic, accountable decision-making with input from clinical, operational, and ethical stakeholders - providing the guardrails necessary for responsible AI deployment and maintaining trust among patients, staff, and our community.

Integrated Response to RFI

HSS appreciates the opportunity to provide input regarding the development, evaluation, and adoption of artificial intelligence (AI) in clinical care. Based on operational experience in healthcare delivery and digital innovation, we offer the following consolidated observations and recommendations.

- 1. Data Readiness and Interoperability:** Data fragmentation and inconsistency remain primary barriers to innovation. Healthcare organizations face challenges identifying necessary data elements, understanding where information resides across systems, distinguishing structured from unstructured data, and accessing complete datasets for training and validation. Greater alignment in data standards, continued focus on interoperability (even within the four walls of a hospital system), and access to shared benchmarking datasets would significantly accelerate responsible AI development and scalability.
- 2. Liability and Accountability Clarity:** Uncertainty regarding responsibility when AI-informed insights influence clinical decisions slows adoption. Clearer guidance on accountability among health systems, clinicians, start-ups, and institutions would reduce risk aversion. This in line with AMA's view that the first step in implementing health AI is to decide and document accountability/oversight and use governance to reduce risk and confusion. Limited safe harbor or risk-sharing models could support responsible pilot programs.
- 3. Evidence-Based Validation and Oversight:** AI tools should be evaluated using evidence-based methods, including performance testing, bias assessment, and workflow evaluation. Post-deployment monitoring should focus on real-world outcomes and model drift. Streamlined, risk-based oversight pathways for non-device AI tools would better align regulation with innovation.
- 4. Human-Centered AI:** Trust is essential. AI must integrate into clinical workflows, reduce burden, and demonstrate measurable improvements in outcomes and efficiency. Adoption must be first led by clinical leadership, in alignment with IT governance, and reimbursement constructs.
- 5. Patient Considerations:** Patients support AI that improves accuracy, timeliness, and personalization of care but express concerns about transparency, privacy, and reduced human interaction. Strong safeguards and clear communication are critical for health systems when implementing AI.
- 6. Incentives and Federal Support:** Payment models should reward validated AI that improves quality and reduces cost. Federal investment in data standards, bias mitigation research, real-world evidence generation, and evaluation frameworks would meaningfully accelerate responsible adoption. This should be paired with consistent federal regulation and legislation to provide clear, uniform guardrails for development, evaluation, and deployment.

Interest and Next Steps

AI holds significant potential to improve clinical quality, operational efficiency, and patient outcomes. Addressing data fragmentation, clarifying liability structures, modernizing evaluation pathways, aligning

reimbursement incentives, and strengthening interoperability will be essential to responsible and scalable adoption. We appreciate the opportunity to provide input and welcome further engagement to support the advancement of safe, effective, and innovative AI use in clinical care. Please contact Stephen X. Hunter, Chief Digital Strategy Officer (HunterS@HSS.edu; 646-797-8748) to coordinate follow-up discussion.